

Introduction

Addictive processes affect people's body, mind, and spirit. When people engage in compulsive, repetitive use of substances or practices like gambling, porn, high-risk sex, and over- or under-eating, it affects their physical health, reconditions their brain, and generates despair. Intimate relationships suffer, families break up, and communities fragment. Collectively, individuals who engage in addictive processes spend billions of dollars each year on alcohol, drugs, porn sites, gambling, groceries, and more. This has resulted in billions more being spent by addiction treatment centers, community mental health centers, and government systems in an attempt to address, prevent, treat, adjudicate, or incarcerate these behaviors (Hart, 2021). In psychotherapy, where do we start?

We suggest a paradigm shift. Rather than asking clients why they engage in compulsive behaviors that lead to negative consequences, we ask how these behaviors or practices help them. Rather than asking why they continue to engage in avoidant, dissociative behaviors that take them out of reality, we ask what they fear would happen if they were completely present. Rather than looking at addictive processes through the lens of mainstream treatment models, which are heavily skewed toward confronting the client and challenging their denial, we suggest engaging the client and healing attachment disruptions and other traumas that activate addictive behaviors in the first place.

We do this with a therapeutic approach called Internal Family Systems (IFS) therapy, which is based on the idea that the psyche has many parts. When one part gets hurt, other parts help by stepping into protective roles. Some protective parts have a proactive role in that they help the client to perform daily tasks and maintain social connections, while others have a reactive role, in that they distract from or soothe emotional pain. The behavior of these protective teams is very different but both have positive intentions. IFS helps clients get curious about these parts so they can discover who they are protecting and why.

When clients come to us, they are often in the grip of a short-term solution (addictive processes) to a long-term problem (disconnection and the sense of shamefulness). They feel discouraged and defeated. However, as IFS therapists, we come to this same meeting with a very different perspective. We know that all parts are deeply committed and well-intentioned. We know that compulsive parts use strategies that may have worked well for past emergencies but are much too costly in the present. We also know these parts want the client to be safe and functional. And we're confident that the options we offer are good. Instead of participating in the chronic power struggles of protective parts, we normalize internal differences, note shared goals, and guide the client to get between parts who are in conflict and help them negotiate.

In this manual, we offer a plan for treating addictive behaviors with IFS by explaining the addictive cycle, describing a series of direct interventions that engage the client without power struggles, and providing safe, clear procedures for addressing underlying emotional pain. We include many experiential exercises and detailed case examples that apply IFS to the wide range of risky behaviors clients use to navigate stress and self-medicate pain. We intend this manual to serve IFS trained therapists who feel anxious when working with addictive processes, as well as any therapist who is comfortable treating addictions but wishes to explore new options.

IFS combines systems thinking with compassionate, inner-focused, experiential psychotherapy. It places the clinician and client on the same gentle road to healing. We know that ordinary people can be loyal, loving, and successful while also being destructive to themselves and others because they're in the midst of an addictive process. The IFS model is tailor-made for that level of complexity.